HEC Forum
https://doi.org/10.1007/s10730-026-09598-5

A Commentary on Ethics Case Reflections as an Ethics laboratory in Clinical Practice

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Received: 19 February 2026 / Accepted: 17 June 2026
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Abstract

This commentary examines the methodological and practical implications of the Ethical Case Reflection sessions reported by Bennesved et al. (2025). We argue that Ethics Case Reflection offers a distinctive contribution to clinical ethics by surfacing both ethically desirable aspirations and problematic attitudes that shape everyday practice. By applying a qualitative descriptive design and conventional content analysis to the original session transcripts, we illustrate how Ethics Case Reflection functions not only as a tool for case-based deliberation but also as an ethics laboratory in which participants explore a broad spectrum of possibilities. We highlight the importance of skilled facilitation for recognizing moments of self-critical reflection, promoting specificity in recommended actions, and supporting attention to both patient-centred and organisational dimensions.

Keywords Clinical ethics · Clinical ethics support · Ethics case reflections · Ethical reflection framework · Facilitation · Professional attitudes · Phronesis

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Published online: 30 June 2026

Introduction

Contemporary healthcare is characterized by rapid technological advancements, frequent organisational change, and high expectations regarding care quality and patient satisfaction. At the same time, demographic shifts and increasing levels of chronic multimorbidity are reshaping healthcare needs, while workforce shortages in many settings are placing additional strain on healthcare provision (European Commission, 2025). These changes have expanded the range and complexity of ethical challenges encountered in everyday clinical practice (Leuter et al., 2018; Ong et al., 2024). Challenges involving autonomy, resource allocation, professional responsibilities, and moral distress experienced by healthcare professionals arise routinely in bedside clinical practice (Beauchamp & Childress, 2019; Rushton, 2016).

Clinical ethics is a practical discipline that offers a structured approach to identifying, analyzing, and resolving ethical issues in patient care. In response to the growing need for ethical support, healthcare systems have implemented clinical ethics support services (CESS) (Hurst et al. u2007; Schildmann et al. 2019a). These are broadly defined as services that support healthcare professionals and institutions in addressing clinical ethical challenges (Dittborn et al., 2022). Reflecting regional, organizational, and philosophical differences, CESS encompasses a wide range of interventions and structured activities designed to facilitate ethical deliberation. Such services are legally mandated in some countries but remain optional in others (Bernard & Fischer, 2025; Pergert et al., 2025). Their scope may vary considerably, supporting healthcare professionals, managers, and patients across different contexts (Slowther et al., 2004).

Clinical ethics support can take multiple forms, including clinical ethics consultation, moral case deliberation, ethical case intervention, ethics reflection groups, ethics committees, ethics rounds, case-based clinical ethics support and ethics facilitation (Fletcher and Siegler 1996; Pergert et al. 2025; Schildmann et al. 2019a). These approaches differ in structure, degree of facilitation, and institutional embedding. In this commentary, we focus specifically on Ethics Case Reflection (ECR) sessions, a structured format of group deliberation in which one or more facilitators guide healthcare professionals in discussing ethically challenging clinical situations relevant to their practice.

ECR in Practice

To clarify its distinctive contribution, ECR can be contrasted with top-down ethics consultations. Top-down ethics consultations typically involve an individual ethics expert or a multidisciplinary ethics committee providing a formaladvisory opinion on a specific case. These processes frequently aim to arrive at a definitive recommendation or "correct" course of action (Rasoal et al., 2017). In contrast, ECR represents a a more participatory, bottom-up approach that emphasizes collective deliberation and the development of participants' moral competence (Lillemoen & Pedersen, 2015; Rasoal et al., 2017).

ECR sessions are usually conducted within the clinical setting and focus not only on resolving specific ethical dilemmas but also on fostering ethical awareness,

improving communication, and strengthening team-based moral competence. They provide a structured space in which nurses, physicians, and other healthcare professionals can articulate concerns, explore alternative perspectives, and collaboratively interpret the values at stake in patient care. ECR has increasingly been adopted as a strategy in clinical practice (Schmitz et al., 2018) and has been associated with enhanced moral awareness, improved interprofessional communication, and greater team cohesion (Crico et al., 2021; Delany et al., 2025; Friedrich et al., 2023; Karlsen et al., 2019). The primary aim of ECR is to support healthcare professionals in managing ethically difficult situations through structured reflection on concrete cases (Lillemoen & Pedersen, 2015).

Effective facilitation is central to this process. Sessions are typically led by individuals with formal and/or practical expertise in ethics and group facilitation. The facilitator is responsible for promoting open dialogue, ensuring adherence to the structured method, and encouraging in-depth reflections within the group.

Despite these potential benefits, the impact of ECR on moral distress remains unclear. While some studies have not been able to demonstrate a consistent reduction in moral distress (Silén et al., 2016; Slowther et al., 2004), this may partly reflect methodological challenges in capturing such effects (Morley et al., 2021). Furthermore, a qualitative evaluation[1] by Frank et al. (2024) suggests that participants experienced increased confidence and reduced stress when dealing with in ethically challenging situations.

The growing implementation of ECR raises important questions about its optimal design and function. These include whether ECR should aim to generate clear recommendations, what competencies facilitators require, and how such practices can best be integrated into routine clinical work (Schmitz et al., 2021; Yoon et al., 2020). Moreover, there is limited understanding of how power dynamics, professional hierarchies, and institutional constraints influence participation and shape the outcomes of ethical deliberations (Mason, 2023).

Relatedly, the role of the facilitator in ECR remains a subject of ongoing debate. Many ECR models advocate a facilitative rather than directive leadership style (de Snoo-Trimp et al., 2020; Schmitz et al., 2021). However, this shift raises questions about how to ensure the ethical quality and rigor of the deliberative process and its outcomes.

Methodology

This commentary is a lengthy secondary analysis of the data from a previously published article (Bennesved et al., 2025) which will henceforth be called ”the primary study”. In this Methodology section we will first briefly present the primary study, then present the rationale for the present commentary, and finally present the methodology of the present commentary.

[1] Drawing on the same empirical material discussed in this commentary.

The Primary Sudy

This article presents a commentary and methodological reflection on a study by Bennesved et al. (2025), which employed ECR as a structured format for ethical deliberation in ambulance care. The study focused analytically on the range of action alternatives proposed by participants, rather than on ethical principles or final decisions. Fourteen ECR sessions were conducted with clinically active ambulance clinicians.

The design of the ECR sessions in the primary study differed from standard practice in several important respects. Rather than using participants' own cases as a starting point, the study introduced a specifically developed video scenario. This approach created a shared and controlled point of departure across all groups, enabling systematic analysis of how the same ethical situation was interpreted and discussed in different ECR sessions. Each group viewed and reflected on a 25-minute video depicting a non-urgent but ethically complex situation involving patient autonomy and potentially impaired decision-making capacity. The scenario concerned an elderly man refusing hospital transport, while his distressed spouse advocated for admission. Importantly, the video concluded before a decision was reached. The case was intentionally left open-ended, allowing participants to define and articulate the ethical challenges themselves.

All sessions followed the Six-Step Model (Lillemoen & Pedersen, 2015), which provides a structured framework for collaborative ethical analysis. In the primary study, the model was implemented through a facilitated group process in which the facilitator summarized participants' contributions in real time. The focus was to investigate participants' comments during the fifth step of the Six-Step Model, i.e. the step where participants were encouraged to explore a broad spectrum of possible actions, ranging from realistic to idealized alternatives (KS, 2022; Lillemoen & Pedersen, 2015). Participants were explicitly invited to generate diverse action alternatives, including unconventional or controversial options, that could be undertaken by any actor within the scenario.

The sessions were facilitated by an experienced clinical ethicist using the facilitating leadership style. Data were analyzed using a combination of summative and conventional content analysis, resulting in 134 unique action alternatives that were grouped into 15 subcategories and four main categories.

Rationale and Aim for this Commentary

By providing ECR sessions based on the same standardized video case, the study design made it possible to examine how ethical challenges and action alternatives were interpreted, emphasized, and reformulated across groups. The insights generated by this ambitious design extend far beyond the specific context of ambulance care. Rather than focusing primarily on clinical outcomes, this commentary seeks to illuminate key mechanisms of ECR as a practice, specifically how ethical reasoning unfolds in group settings, how facilitation shapes reflective processes, and how the format supports the articulation of tacit professional knowledge. By moving beyond context-specific findings, this analysis contributes to a deeper understanding of both

commonalities and variations in ECR practice and highlights central mechanisms that may inform its further development and implementation. Therefore, The aim of this commentary is to explore the methodological utility of ECR.

Methodology of the Present Commentary

This commentary draws directly on both the findings and the raw empirical data from Bennesved et al. (2025), where the original methodology is described in detail. A renewed analysis of the original transcripts, in conjunction with the previously developed category system, provided a novel methodological perspective. This approach shifts attention toward what the content of the discussions and the diversity of the proposed action alternatives reveal about ECR as a practical method. The study design, with its shared point of departure, enables a detailed examination of how ethical challenges and action alternatives are interpreted, emphasized, and reformulated across groups. The insights generated extend beyond the specific context of ambulance care.

Discussion

ECR Do Not Merely Report on Reality but also Articulate Ideals

The discussions reported in this commentary demonstrate how the ECR format foster a clear commitment among participants to contextualized ethical reasoning. As argued by O'Neill (2002), ethical reasoning must remain grounded in its context rather than detach itself from the lived experiences and realities of care. By paying close attention to the details of the video case, and by moving back and forth between these details and the proposed action alternatives, the ECR structure explicitly encouraged a highly context-sensitive ethical discussion. Consequently, participants consistently anchored their deliberations in the concrete circumstances of the case, successfully integrating relational, social, and institutional factors. This context-sensitive orientation enhances the practical relevance of ECR and aligns with contemporary calls for situated ethics in healthcare.

Previous studies have shown that when healthcare professionals talk about ethics, they do not necessarily describe how they act in practice, but rather how they believe one *ought* to act. In other words, they express ethical ideals rather than describe reality (Bærøe & Henden, 2020; Nijsingh et al., 2020; Sisk et al., 2020). Against this backdrop, it is interesting to note that the insights presented in this commentary show how the participants' discussions encompass both the realistic and the idealized in the ECR.

On the realistic side, some ECR sessions generated action alternatives that corresponded closely to existing organizational structures and professional mandates. Others alternated between realistic possibilities and imagined scenarios that participants recognized as unattainable under current conditions. For instance, the desire for patients to have access to an acute consultation with a physician familiar with their case was repeatedly described as an idealized example of care, clinically desirable yet organizationally unavailable. This illustrates how ECR articulate the professional

judgement required to navigate the gap between ethical ideals and what is feasible within prevailing resource constraints (Weston, 2007). Two of the authors of this commentary, who are themselves experienced ambulance clinicians, can confirm that while some proposed action alternatives were clearly in excess of the possible, others balanced finely within that zone where ethical aspiration and organizational reality overlap. In this sense, the ECR sometimes revealed promising new routes forward, by recommending feasible yet currently underutilized alternatives, such as arranging for a district nurse or a general practitioner to conduct a home visit within the next 24 h.

Hence, the presence of idealized or seemingly unrealistic elements in ECR discussions, elements that do not fully reflect the constraints of everyday clinical practice, does not diminish their value. Beyond generating concrete solutions, the articulation of shared ideals may serve as a source of inspiration and a catalyst for change. This perspective aligns with the concept of *phronesis* (Bontemps-Hommen et al., 2019), in which moral learning is fostered not only through the resolution of immediate problems, but also through reflection on moral exemplars and an awareness of the gap between current practice and optimal care.

This tension between routine practice and aspirational ideals is illustrated by a participant who described the difficulty of moving from standardized medical assessment to focus more on the patient: *"You want to take a history and check vital parameters… And then you come to the next stage where it becomes this person-centred care where [you ask] 'what is most important to you?'… but my experience was that you don't always get there but instead choose to interrupt [the assessment] earlier, and then things can go wrong."*

Importantly, this reflection does not reject the value of established medical routines as they remain ethically essential especially in acute situations. Instead, it highlights the clinical challenge of shifting perspectives when a situation proves to be non-urgent. While such reflections do not in themselves constitute *phronesis*, they nevertheless demonstrate the kind of heightened moral awareness that ECR can facilitate.

By making explicit the tension between technical-medical routines and patient-centred ideals, ECR creates a space in which clinicians can critically examine their practice and negotiate shared aspirations for care. In doing so, it enables reflection on what ought to be strived for, even in the face of prevailing organizational and institutional constraints.

Our broader experience with ECR, supported by the analysis of the sessions in the primary study, suggests that the interplay between realistic and idealistic reasoning is a commonplace feature of "good ECR". In our review of the recorded material, we observed that idealistic perspectives emerged most clearly as participants became less inhibited and more engaged in the discussion. The audio data indicates that participants displayed heightened engagement when describing unconventional yet potentially valuable action alternatives. These creative phases of ECR seem to constitute particularly significant moments in the reflective process. By temporarily suspending immediate practical constraints, participants can articulate professional aspirations, expressing what ought to be pursued, rather than what is currently feasible. Despite its apparent importance, this tension between the ideal and the achievable remains underexplored in the clinical ethics literature.

ECR facilitators may play a crucial role in mediating the tension between idealism and realism. Before transition from stage V to stage VI in the Six-Step Model (or comparable phases in other frameworks), facilitators could systematically review proposed action alternatives with explicit attention to their feasibility in practice. While options that are not immediately actionable should be clearly identified, their potential relevance for future organizational development or practice improvement may nevertheless warrant further exploration.

It should be noted that none of the ECR examined in this commentary centred predominantly on idealistic reasoning, suggesting that the ECR format keeps the discussion anchored in clinical reality. Nonetheless, even while we encourage inviting the ideal beside the realistic, the balance between the poles must be kept. The literature on clinical ethics consultation emphasizes that practical applicability is a key criterion for ECR quality (Haltaufderheide et al., 2022). Thus, while imaginative proposals can be valuable, ECR facilitators must ensure that participants remain attentive to what can meaningfully inform clinical practice. This balancing act is illustrated in a sequence where participants explored a combination of clinical interventions (e.g., assessing vital parameters, contacting primary care) alongside a light-hearted, relational suggestion (e.g., offering a hug) to ease the transport decision. Although the group momentarily engaged with this imaginative alternative, the facilitator intervened to critically assess its practical applicability considering the patient's firm refusal: *"As an ethicist, I'm still thinking that it's not certain he will be satisfied just with a hug. I can imagine that he still won't want to go to the hospital."*

By problematizing the utopian assumption that relational warmth alone could resolve a complex refusal, the facilitator effectively anchored the discussion in clinical reality. This sequence demonstrates that, while playful or unconventional alternatives should be encourage during stage V, they must also be subjected to systematic critical scrutiny. Facilitators play a key role in this process: they need to create space for creative exploration while simultaneously acting as a pragmatic counterweight, ensuring that final reflections remain responsive to both patient autonomy and the practical constraints of clinical care.

ECR May Uncover Problematic Working Habits

There is a growing interest in developing competencies and approaches that more effectively support patient autonomy (Botana Gronek et al., 2025; Bremer & Holmberg, 2020). In this context, ECR appears to hold considerable potential for revealing values and attitudes that threaten patient autonomy or other ethical values. Rather than merely providing a forum for articulating ethical ideals, ECR also uncovers attitudes that stand in tension with those ideals. To illustrate this, the following section explores in depth how the ECR format enabled participants to discuss ethically objectionable working habits. The findings show that ACs are highly aware of negative views within their own rank toward non-urgent medical situations and, by implication, toward patients resembling the elderly man portrayed in the video. This was evident in comments such as: *"We still hear that these [non-urgent patients] are not our patients to treat."* Such negative attitudes toward non-urgent cases and so-called 'rubbish calls' align with previous research in ambulance care (Porter et al., 2007).

By uncovering attitudes within the workforce which are in opposition to accepted ethical ideals in healthcare, an opportunity is opened to discuss attitudes and norms in the workforce that may prove problematic in relation to both professional and societal ideals. Our experience is that the spirit of playful discovery which permeated these ECRs may be a very fruitful setting for such discussions. By simultaneously revealing both ethical ideals and problematic attitudes, ECRs highlight the complex ethical realities in clinical work, where expressed principles do not always translate into everyday actions. By enabling reflection across a full spectrum of perspective, ranging from the acceptable and realistic to those that are the less ethically favourable yet still grounded in practice, ECR enhances its value as a method capable of capturing a nuanced picture of professional life. In this way, ECR can function as a bridge between ethics *in principle* and ethics *in practice*, supporting the development of professional self-awareness and ethical competence (Nimo et al., 2025). This bridging function becomes particularly evident when participants reflect on structural barriers within formal referral pathways. During the ECR sessions, ambulance clinicians highlighted their limited ability to arrange primary care follow-up, especially when such services are unavailable outside regular hours. They emphasized their lack of formal mandate to establish alternative care pathways, stating: *"We can't write anything referral-like*," and pointing to resistance from other parts of the healthcare system: *"And it's rare that anyone wants to accept [the patient] at the primary care centre at those times."*

When ethical judgments cannot be translated into action due to structural constraints, such as the inability to secure an appropriate level of care, clinicians are often forced into a stark binary choice revealing a structural misalignment between ethical judgment and available courses of action.

Implications of Great Variation Between ECR

The empirical material shows that, when guided by a facilitating rather than directive leader, there was considerable variation across ECR sessions, even when the case was discussed. Differences in what groups attended to, and what they overlooked, suggest that ECR interacts not only with individual reasoning but also with broader organizational and systemic dimensions of ethical practice. This observation provides an important starting point for considering the wider implications of such variation.

While some commentators emphasize that the aims of ECR extend beyond individual learning, this aspect is not consistently highlighted. For example, Schmitz et al. (2018) identify organizational objectives, such as strengthening a reflective climate within healthcare institutions. Findings from the primary study discussed in this commentary indicate that some, but not all, groups engaged with such organizational dimensions. When discussing the same video case of the elderly man refusing transport, certain groups focused almost exclusively on micro-level aspects of the patient encounter, such as communication strategies or how to support the distressed spouse. In contrast, other groups broadened the discussion to include meta-level factors such as team attitudes, as well as macro-level systemic conditions, like the lack of alternative care pathways and the limited availability of primary care services.

This contrast illustrates that, in the absence of explicit facilitator prompts, ECR groups may overlook structural factors that shape ethical dilemmas. If systemic issues are consistently neglected, ECR risks falling short of its broader organizational aims (Schmitz et al., 2018), functioning primarily as a tool for individual problem-solving rather than as a means of strengthening institutional reflective capacity.

The fact that only some groups adopt this broader perspective resonates with Silva et al. (2008), who caution that Clinical Ethics Committees may become overly focused on individual patient cases at the expense of structural concerns. ECR has the potential to bridge between the micro-macro divide, but likely only if facilitators actively encourage participants to incorporate organizational considerations into their deliberations. Without such encouragement, many groups may remain confined to the immediate clinical context.

This raises an important methodological question for ECR: should facilitators adhere strictly to a non-directive style, even if this risks leaving key dimensions underexplored? Or should they take a more active role in prompting reflection on organizational factors, such as resource constraints, policy frameworks, accountability structures, and interprofessional coordination? A third alternative would be to formalize such prompts within ECR models themselves. Currently, no explicit prompts of this kind are included in, for example, the Six-Step Model. Introducing them could allow facilitators to maintain a primarily facilitating role while still ensuring that participants attend to the systemic forces shaping ethical practice.

Encouraging participants to generate a wide range of action alternatives is consistent with recommendations for the Six-Step Model (KS, 2022). In the present study, each group produced at least eight alternatives, resulting in a rich overall dataset. However, the use of multiple ECR sessions based on the same case revealed that not only organizational aspects but also directly patient-related challenges may be omitted. For instance, the ethically significant theme of potentially problematic attitudes within the clinical team was explored in only four of the 14 groups.

This omission highlights a potential limitation of the facilitative leadership style: it cannot ensure comprehensive coverage of all ethically relevant dimensions within a single session. Indeed, this reflects a broader limitation of bottom-up, group-centred approaches (Rasoal et al., 2017).

ECR as 'Ethics Laboratory'

Reflecting on the primary findings reveals that ECR functions not merely as a forum for identifying ethically justified actions, but also as a space for exploring the broader landscape of possible reasoning and practice. This interpretive lens provides a useful point of departure for examining how ECR operates as an 'ethics laboratory'. The empirical data from the ECR sessions clearly illustrates how this laboratory environment allows participants to step away from daily operational routines where ethical challenges often, as one participant noted, *"run on autopilot."* The sessions provide a reflective space where it is valued as *"good to get into it again and discuss openly"* because these are *"difficult questions and very, very broad."* The practical output of this empirical reflection is further highlighted when participants move from abstract ethics to formulate highly context-sensitive actions tailored to the patient's immedi-

ate situation; as one participant emphasized, doing what is best *"can be anything from walking the dog first before we go to the hospital."*

Part of the observed variation in proposed action alternatives can be understood through what may be described as a *building-block approach*. Rather than moving from one distinct option to another, participants frequently started with a basic idea such as *"gather more information"* and interactively honed this idea to become *"gather more information from the patient"* or "*gather more information from relatives or other healthcare professionals."* Such contextually sensitive refinements enhanced the practical relevance of action alternatives by transforming abstract suggestions into more actionable steps.

At the same time, several action alternatives remained underspecified. With underspecified alternatives such as *"provide care support"* it was likely unclear to everyone in the ECR what should be done and by whom. If actionability of recommendations is seen, as we think it should be, as a core quality criterion of ECR, the high frequency of underspecified alternatives marks an area for methodological improvement. Clear and specified formulation of actions is particularly important for stage VI (recommendation) of the Six-Step Model, which can be strengthened by clarity already at earlier stages in the process. Also, we believe that generating a plethora of highly specified alternatives in stage V (action alternatives) is valuable in itself as it showcases the wealth of options. While options that are unsuitable as immediate recommendations may still provide valuable inspiration for future clinical scenarios, the effort to move from such general ideas toward concrete specification must be balanced against the time constraints of ECR sessions. As Reiter-Theil (2000) notes, an excessive focus on detail may prolong deliberation or risk shifting the group's attention disproportionately toward practicalities at the expense of ethical reflection.

The plurality of action alternatives evidenced in the primary study further illustrates how the Six-Step Model supports a pluralistic mode of ethical reasoning, in contrast to the dilemma-based 'either/or' framing common in much of the ethics literature (Stolper et al., 2016). When given sufficient time and conceptual space, participants frequently combined multiple action alternatives to address several values simultaneously, for example, by supplementing the option of leaving the patient at home with follow-up measures such as arranging a visit or supporting a spouse. We do not hereby intend that the Six-Step Model is superior (Schildmann et al. 2019b), but rather we suggest that ECR using any model may benefit from encouraging participants to explore a plurality of potentially additive courses of action.

We argue that ECR should not be viewed solely as a method for identifying a single 'correct' course of action. Rather, it can be conceptualised as an ethics laboratory (Mumford et al., 2010) in which participants are encouraged to explore a wide spectrum of possible ways of reasoning and acting, including options that may be less appropriate, counterintuitive, or initially perceived as unfeasible. Making this full range visible creates opportunities for deeper reflection on values, boundaries, and implicit norms that inform professional judgment. Within such a laboratory-like setting, the role of the group facilitator is crucial (Frank et al., 2024). Skilled leadership enhances the quality of deliberation by promoting argumentative variety, critical engagement, and moral focus, and by ensuring that all professional perspectives are considered, including those less dominant in hierarchical team structures (Karlsen et

al., 2019; Kremer et al., 2025). Group facilitators also help maintain a structured process, foster a psychologically safe climate, and guide participants in clarifying and critically examining underlying values and assumptions (Bartholdson et al., 2018; Delany et al., 2025; Karlsen et al., 2019; Kremer et al., 2025).

Implications for the ECR Facilitator

Reflecting on the findings from the primary study highlights the crucial role of the facilitator in recognising and responding to comments that open opportunities for self-critical reflection on potentially problematic working habits. The emergence of such perspectives during the sessions illustrates the capacity of ECR to surface ethical tensions and practical challenges that likely extend beyond the specific case discussed. Consequently, this potential for deep reflection places clear demands on the facilitation process.

When ethically troubling practices or habitual ways of working are mentioned only briefly, there is a risk that these challenges may pass without scrutiny and thereby become inadvertently normalised. To counteract this risk, facilitators may need to actively encourage participants to articulate and explore such patterns more explicitly. This facilitative responsibility, supporting deeper self-critical dialogue when it arises, has received limited attention in the existing literature on ECR facilitation, despite its potential importance for realising the method's aims. Consequently, addressing these moments may require the facilitator to adopt a more directive stance than is commonly associated with ECR methodologies. More guidance from the facilitator can increase cognitive demands on participants and may challenge the Socratic ideal that underpins many approaches to ECR (Stolper et al., 2016). Indeed, if the leader is merely facilitating rather than stepping in and steering the discussion, ECR may potentially go in many different directions and omit relevant perspectives, reach no conclusion, or arrive at ethically unsound conclusions (Bolton, 2023; Schmitz & Duewell, 2022; Watson & Guidry-Grimes, 2018). Balancing this need for directive intervention with the aim of fostering autonomous moral reasoning thus becomes a key practical and pedagogical consideration for ECR facilitators. This balancing act significantly increases the cognitive demands on the facilitator, who must remain highly alert to implicit cues within the group. One possible way to navigate this tension is using in-house ECR facilitated by members of clinical ethics committees. Such facilitators may bring not only ethical expertise but also organizational legitimacy, which can support sustained and systematic critical reflection across sessions. This approach may promote greater methodological consistency while still preserving the open, exploratory character that defines ECR. At the same time, we are aware of the valid concerns with being an ECR facilitator in the clinical context where one works (Pergert et al., 2025).

Conclusion

This commentary shows that ECR offers a structured setting in which healthcare professionals can examine the practical considerations as well as and the value commitments that shape their work. ECR may reveal ethically grounded options alongside assumptions, uncertainties, and organizational pressures that influence everyday judgment. This combination of factual reasoning and exploratory thinking underscores the value of ECR as a space for examining tensions and broadening perspectives.

Differences across groups demonstrate that outcomes depend on how discussions unfold and how they are guided by the ECR facilitator. Certain ethically significant themes, such as team attitudes or systemic constraints, were addressed unevenly in a large study with a series of ECR, highlighting the importance of attentive leadership that can clarify vague proposals, recognize emerging concerns, and gently introduce broader contextual factors when needed.

By encouraging explicit moral reflection and supporting nuanced dialogue, ECR may contribute to more deliberate and responsible clinical decision-making. In summary, ECR is best understood as a dynamic and exploratory process - an ethics laboratory - in which facilitation is essential for cultivating open dialogue, broadening the scope of ethical reasoning, and supporting the development of ethically defensible care practices. Attention to both patient-centred and organizational dimensions is vital if ECR is to fully realize its potential as a situated ethical practice.

Author Contributions A.B. and J.B. made substantial contributions to the conception and writing of the commentary. A.Br. provided critical revisions and contributed to the content of the manuscript. All authors approved the final version to be published and agree to be accountable for all aspects of the work.

Funding Open access funding provided by Linnaeus University.

Data Availability No datasets were generated or analysed during the current study.

Declarations

Competing Interests The authors declare no competing interests.

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